2.5 By 2020, maintain the genetic diversity of seeds, cultivated plants and farmed and domesticated animals and their related wild species, including through soundly managed and diversified seed and plant banks at the national, regional and international levels, and promote access to and fair and equitable sharing of benefits arising from the utilization of genetic resources and associated traditional knowledge, as internationally agreed

2.a Increase investment, including through enhanced international cooperation, in rural infrastructure, agricultural research and extension services, technology development and plant and livestock gene banks in order to enhance agricultural productive capacity in developing countries, in particular least developed countries

2.b Correct and prevent trade restrictions and distortions in world agricultural markets, including through the parallel elimination of all forms of agricultural export subsidies and all export measures with equivalent effect, in accordance with the mandate of the Doha Development Round

2.c Adopt measures to ensure the proper functioning of food commodity markets and their derivatives and facilitate timely access to market information, including on food reserves, in order to help limit extreme food price volatility

Goal 3. Ensure healthy lives and promote well-being for all at all ages

3.1 By 2030, reduce the global maternal mortality ratio to less than 70 per 100,000 live births

3.2 By 2030, end preventable deaths of newborns and children under 5 years of age, with all countries aiming to reduce neonatal mortality to at least as low as 12 per 1,000 live births and under-5 mortality to at least as low as 25 per 1,000 live births

3.3 By 2030, end the epidemics of AIDS, tuberculosis, malaria and neglected tropical diseases and combat hepatitis, water-borne diseases and other communicable diseases

3.4 By 2030, reduce by one-third premature mortality from non-communicable diseases through prevention and treatment and promote mental health and well-being

3.5 Strengthen the prevention and treatment of substance abuse, including narcotic drug abuse and harmful use of alcohol

3.6 By 2020, halve the number of global deaths and injuries from road traffic accidents

3.7 By 2030, ensure universal access to sexual and reproductive health-care services, including for family planning, information and education, and the integration of reproductive health into national strategies and programmes

3.8 Achieve universal health coverage, including financial risk protection, access to quality essential health-care services and access to safe, effective, quality and affordable essential medicines and vaccines for all

3.9 By 2030, substantially reduce the number of deaths and illnesses from hazardous chemicals and air, water and soil pollution and contamination

3.a Strengthen the implementation of the World Health Organization Framework Convention on Tobacco Control in all countries, as appropriate

3.b Support the research and development of vaccines and medicines for the communicable and non-communicable diseases that primarily affect developing countries, provide access to affordable essential medicines and vaccines, in accordance with the Doha Declaration on the TRIPS Agreement and Public Health, which affirms the right of developing countries to use to the full the provisions in the Agreement on Trade-Related Aspects of Intellectual Property Rights regarding flexibilities to protect public health, and, in particular, provide access to medicines for all

3.c Substantially increase health financing and the recruitment, development, training and retention of the health workforce in developing countries, especially in least developed countries and small island developing States

3.d Strengthen the capacity of all countries, in particular developing countries, for early warning, risk reduction and management of national and global health risks

Goal 4. Ensure inclusive and equitable quality education and promote lifelong learning opportunities for all

4.1 By 2030, ensure that all girls and boys complete free, equitable, and quality primary and secondary education leading to relevant and effective learning outcomes

4.2 By 2030, ensure that all girls and boys have access to quality early childhood development, care and pre-primary education so that they are ready for primary education

4.3 By 2030, ensure equal access for all women and men to affordable and quality technical, vocational and tertiary education, including university

4.4 By 2030, substantially increase the number of youth and adults who have relevant skills, including technical and vocational skills, for employment, decent jobs and entrepreneurship

4.5 By 2030, eliminate gender disparities in education and ensure equal access to all levels of education and vocational training for the vulnerable, including persons with disabilities, indigenous peoples and children in vulnerable situations

4.6 By 2030, ensure that all youth and a substantial proportion of adults, both men and women, achieve literacy and numeracy

4.7 By 2030, ensure that all learners acquire the knowledge and skills needed to promote sustainable development, including, among others, through education for sustainable development and sustainable lifestyles, human rights, gender equality, promotion of a culture of peace and non violence, global citizenship and appreciation of cultural diversity and of culture's contribution to sustainable development

4.a Build and upgrade education facilities that are child, disability and gender sensitive and provide safe, non-violent, inclusive and effective learning environments for all

4.b By 2020, substantially expand globally the number of scholarships available to developing countries, in particular least developed countries, small island developing States and African countries, for enrolment in higher education, including vocational training and information and communications technology, technical, engineering and scientific programmes, in developed countries and other developing countries

4.c By 2030, substantially increase the supply of qualified teachers, including through international cooperation for teacher training in developing countries, especially least developed countries and small island developing States

Chapter 3

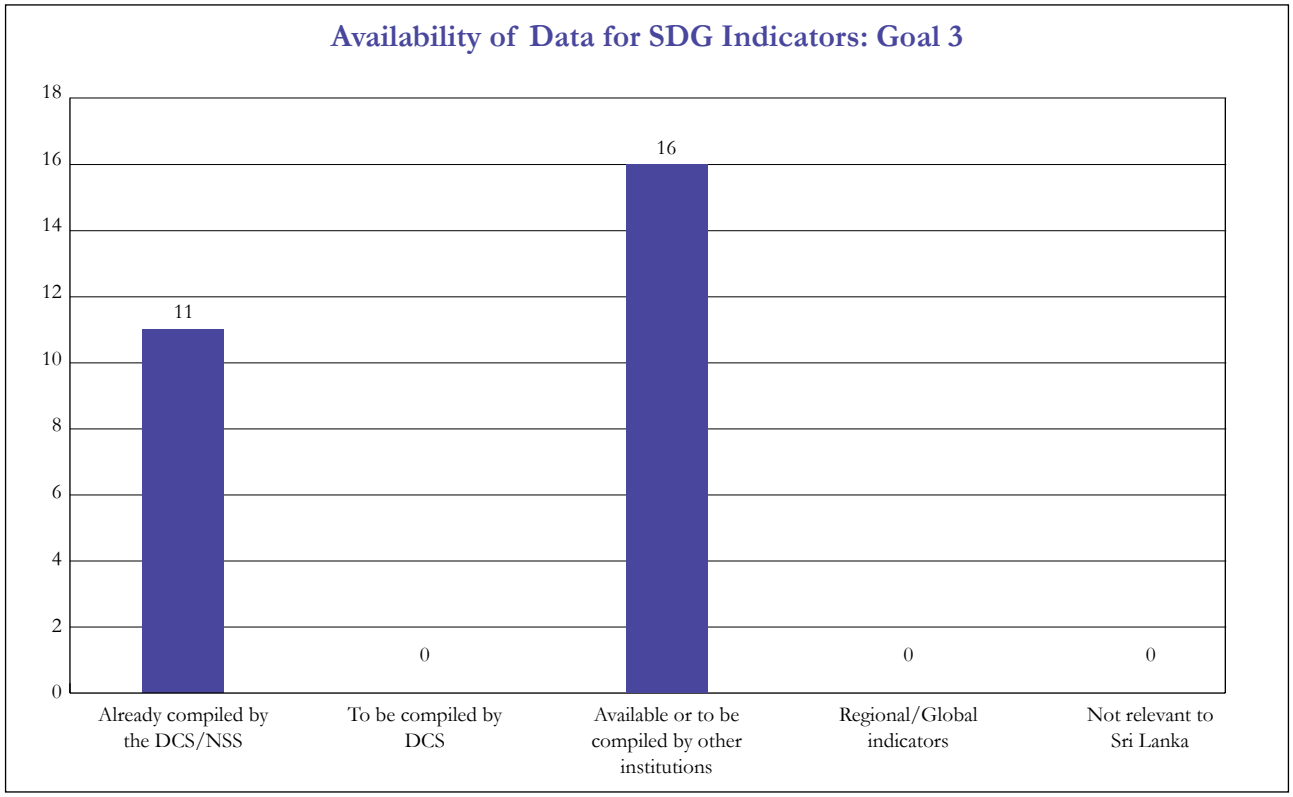
GOAL 3: ENSURE HEALTHY LIVES AND PROMOTE WELL-BEING FOR ALL AT ALL AGES

This Chapter provides details on the SDG 3 with respect to the Sri Lankan context.



SDG 3 includes 13 targets and 27 indicators. Out of the 27 indicators, data are already compiled by the DCS for 11 indicators.

The following chart shows the number of indicators for which baseline data are already compiled by DCS, to be compiled by DCS, available or to be compiled by other institutions, regional or global indicators and indicators not relevant to Sri Lanka.



Definitions for each indicator, data already available and compiled by the DCS and details on indicators for which data could be made available are presented in the next section.

3.1 DATA ALREADY COMPILED BY DCS

3.1.1 Target 3.1: By 2030, reduce the global maternal mortality ratio to less than 70 per 100,000 live births.

Indicator 3.1.1: Maternal mortality ratio (Tier 2)

UNSD Definition:

The annual number of female deaths from any cause related to or aggravated by pregnancy or its management (excluding accidental or incidental causes) during pregnancy and childbirth or within 42 days of termination of pregnancy, irrespective of the duration and site of pregnancy per 100,000 live births, for a specific time period.

DCS/NSS Definition:

The number of female deaths registered in a given year from any cause related to or aggravated by pregnancy or its management (excluding accidental or incidental causes) during pregnancy and childbirth or within 42 days of termination of pregnancy, irrespective of the duration and site of pregnancy per 100,000 live births registered in the same year.

Baseline Data:

Maternal mortality ratio

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	Per 100,000 live births	26.8	2013	Report on Vital Statistics, RGD

Indicator 3.1.2: Proportion of births attended by skilled health personnel (Tier 1)

UNSD Definition:

The percentage of live births attended by skilled health personnel during a specific time period, generally up to the past five years.

DCS Definition:

Same as UNSD definition.

Baseline Data:

Proportion of births attended by skilled health personnel

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	%	99.5	2016	DHS, DCS

3.1.2 Target 3.2: By 2030, end preventable deaths of newborns and children under 5 years of age, with all countries aiming to reduce neonatal mortality to at least as low as 12 per 1,000 live births and under-5 mortality to at least as low as 25 per 1,000 live births.

Indicator 3.2.1: Under-five mortality rate[6] (Tier 1)

UNSD Definition:

The probability of a child born in a specific year or period dying before reaching the age of 5 years per 1000 live births.

DCS/NSS Definition:

Number of deaths before reaching age 5 and registered in a given year per 1000 live births registered in the same year.

Baseline Data:

Under-five mortality rate

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	**Per 1,000 live births**	**9.4**	**2014**	Report on Vital Statistics, RGD

[6] Provisional Data

Indicator 3.2.2: Neonatal mortality rate[7] (Tier 1)

UNSD Definition:

The neonatal mortality rate is the probability that a child born in a specific year or period will die during the first 28 completed days of life if subject to age - specific mortality rates of that period, expressed per 1,000 live births.

DCS/NSS Definition:

Number of deaths before completing the 28 days of life registered in a given year per 1,000 live births registered in the same year.

Baseline Data:

Neonatal mortality rate (Tier 1)

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	Per 1,000 live births	5.6	2014	Report on Vital Statistics, RGD

[7] Provisional Data

3.1.3 Target 3.4: By 2030, reduce by one third premature mortality from non-communicable diseases through prevention and treatment and promote mental health and well–being

Indicator 3.4.1: Mortality rate attributed to cardiovascular disease, cancer, diabetes or chronic respiratory disease (Tier 2)

UNSD Definition:

Mortality rate attributed to cardiovascular disease, cancer, diabetes or chronic respiratory disease. Probability of dying between the ages of 30 and 70 years from cardiovascular diseases, cancer, diabetes or chronic respiratory diseases, defined as the per cent of 30-year-old-people who would die before their 70th birthday from cardiovascular disease, cancer, diabetes, or chronic respiratory disease, assuming that s/he would experience current mortality rates at every age and s/he would not die from any other cause of death (e.g., injuries or HIV/AIDS). This indicator is calculated using life table methods.

Cardiovascular disease, cancer, diabetes or chronic respiratory diseases: ICD-10 underlying causes of death I00-I99, COO-C97, E10-E14 and J30-J98.

DCS/NSS Definition:

NSS calculates a proxy indicator; Number of deaths attributed to cardiovascular disease, cancer, diabetes or chronic respiratory disease between aged 30-69 years and registered during a given year per 10,000 population of aged 30-69 years during the same period.

Cardiovascular disease, cancer, diabetes or chronic respiratory diseases: ICD-10 underlying causes of death I00-I99, COO-C97, E10-E14 and J30-J98.

Baseline Data:

Mortality rate attributed to cardiovascular disease, cancer, diabetes or chronic respiratory disease

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total Deaths	**Per 10,000 population of aged 30-69**	**31.9**	**2013**	Report on Vital Statistics, RGD
Cardiovascular disease	Per 10,000 population of aged 30-69	15.3	2013	-do-
Cancer		8.7	2013	-do-
Diabetes		5.0	2013	-do-
Chronic respiratory disease		3.0	2013	-do-

Indicator 3.4.2: Suicide mortality rate (Tier 2)

UNSD Definition:

The number of suicide deaths in a year, per 100, 000 population

DCS/NSS Definition

The number of suicide cases reported to the Police per 100,000 population of the country during the calendar year.

Baseline Data:

Suicide mortality rate

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	**Per 100,000 population**	**14.3**	**2016**	Sri Lanka Police
By Sex				
Male	Per 100,000 population	22.8	2016	-do-
Female		6.3	2016	-do-

3.1.4 Target 3.6: By 2020, halve the number of global deaths and injuries from road traffic accidents

Indicator 3.6.1: Death rate due to road traffic injuries (Tier 1)

UNSD Definition:

Death rate due to road traffic injuries as defined as the number of road traffic fatal injury deaths per 100,000 population.

DCS/NSS Definition:

Number of deaths due to road traffic fatal injuries, reported to the Police per 100,000 population of the country during the calendar year.

Baseline Data:

Death rate due to road traffic injuries

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	**Per 100,000 population**	**14.2**	**2016**	Sri Lanka Police
By Sex				
Male	Per 100,000 population	24.5	2016	-do-
Female		4.6	2016	-do-

3.1.5 Target 3.7: By 2030, ensure universal access to sexual and reproductive health-care services, including for family planning, information and education, and the integration of reproductive health into national strategies and programmes

Indicator 3.7.1: Proportion of women of reproductive age (aged 15-49 years) who have their need for family planning satisfied with modern methods (Tier 1)

UNSD Definition:

The percentage of women of reproductive age (15-49 years) who desire either to have no (additional) children or to postpone the next child and who are currently using a modern contraceptive method.

DCS Definition:

Percentage of women of reproductive age (15-49 years) who are sexually active and who have their need for family planning satisfied with modern methods.

Baseline Data:

Proportion of women of reproductive age (aged 15-49 years) who have their need for family planning satisfied with modern methods

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	%	74.2	2016	DHS, DCS

Indicator 3.7.2: Adolescent birth rate (aged 10-14 years; aged 15-19 years) per 1,000 women in that age group (Tier 2)

UNSD Definition:

Annual number of births to females aged 10-14 or 15-19 years per 1,000 females in the respective age group.

DCS Definition:

Annual number of births to women aged 15-19 years per 1,000 women in that age group.
Note: It is also referred to as the age-specific fertility rate for women aged 15-19 years.

Baseline Data:

Adolescent birth rate (aged 10-14 years; aged 15-19 years) per 1,000 women in that age group

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Total	Per 1,000 women in 15-49 age group	36.0	2012	CPH, DCS

3.1.6 Target 3.c: Substantially increase health financing and the recruitment, development, training and retention of the health workforce in developing countries, especially in least developed countries and small island developing States.

Indicator 3.c.1: Health worker density and distribution [All Medical Officers, Specialist Medical Officers (curative), Nurses, Midwives (Public Health+Hospital), Public Health Inspectors] (Tier 1)

UNSD Definition:

Density of health workers per 1,000 population.

Traditionally, this indicator has been estimated using 2 measurements: density of physicians and density of nursing and midwifery personnel (both per 1,000 population). For each country the same or the closest year was used to the extent available. In the context of the SDG agenda, the dataset will be expanded to physicians, nursing personnel, midwifery personnel, dentists, pharmacists and progressively move to cover all health cadres.

DCS/NSS Definition:

Density of health workers by cadre per 100,000 population, including generalist medical practitioners, specialist medical practitioners (surgeons, anaesthetists, obstetricians, emergency medicine specialists, cardiologists, paediatricians, psychiatrists, ophthalmologists, gynaecologists etc.), nursing and midwifery professionals, traditional and complementary medicine professionals, among others.

Baseline Data:

Health worker density and distribution [All Medical Officers, Specialist Medical Officers (curative), Nurses, Midwives (Public Health+Hospital), Public Health Inspectors]

Disaggregation level	Baseline			
	Unit	Value	Year	Data Source, Agency
Medical Officers	Per 100,000 population	87.0	2015	Annual Health Bulletin-MOH
Specialist Medical Officers (Curative)[b]		8.5	2015	-do-
Specialist Medical Officers (Preventive[b])		0.5	2015	-do-
Specialist Medical Officers [b]		9.0	2015	-do-
Nurses		202.3		-do-
Midwives (Public Health+ Hospital)		42.0	2015	-do-
Public Health Midwives		28.8	2015	-do-
Hospital Midwives		13.2	2015	-do-
Public Health Inspectors		7.7	2015	-do-

a. Provisional
b. Excludes administrative grade

3.1.7 Target 3.8 Achieve universal health coverage, including financial risk protection, access to quality essential health-care services and access to safe, effective, quality and affordable essential medicines and vaccines for all.

Indicator 3.8.2: Number of people covered by health insurance or a public health system per 1,000 population[8] (Tier 2)

UNSD Definition:

Not available.

Sri Lanka has a universal healthcare system that provides free healthcare to all citizens.

[8] The whole population is covered by the public health system.